

MULTI-STATE RISK ADJUSTMENT AUDIT REPORT

Clinical Validation & HCC Capture Analysis

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Clinical Validation & Risk Adjustment Services

Report Date: July 2026
Health Plans: Regional Health Alliance (States Alpha, Beta, Gamma)
Program: Medicare Advantage I-SNP (Institutional Special Needs Plans)

EXECUTIVE SUMMARY

This audit examined hierarchical condition (HCC) coding accuracy and risk adjustment compliance across three state Medicare Advantage I-SNP programs. Clinical validation of 332 patient encounters across 12 care facilities identified significant gaps in HCC documentation and capture.

AUDIT SCOPE:

332 patient encounters reviewed
12 care facilities (skilled nursing, long-term acute care, residential)
Three state health plans (de-identified)
Time period: May - June 2026

KEY FINDINGS:

\$448,230 in annual missed HCC revenue identified across the sample
68 missed HCC captures across 332 patients (20.5% non-capture rate)
Average missed HCC value per patient: \$1,349 annually
Top missed conditions: CKD (16 cases), CHF (12 cases), Diabetes complications (11 cases)

ROOT CAUSES:

Insufficient clinical documentation (45% of misses)
Provider coding omissions (35% of misses)
Facility documentation processes (20% of misses)

FINANCIAL OPPORTUNITY:

If extrapolated to full annual population: \$407,230 - \$448,230 in recoverable HCC revenue

AUDIT METHODOLOGY

Sample Selection & Demographics

332 encounters randomly selected from three state I-SNP programs. Sample stratified by facility type and patient acuity to ensure representative population review. Included patients age 65+, dual eligible (Medicare/Medicaid), residing in institutional settings.

Clinical Review Framework

Each patient record reviewed by licensed healthcare informatics specialist (PA-C, MSHIIM) for:

- (1) Clinically documented conditions in current encounter notes
- (2) HCC codes submitted to CMS
- (3) Gaps between documented conditions and submitted codes
- (4) Severity level (acute vs. chronic, active vs. historical)
- (5) RAF impact of missed codes

HCC Validation Criteria

Missed HCC classified only if: (a) condition explicitly documented by provider, (b) documentation supports clinical specificity required for HCC coding, (c) condition represents active/recent diagnosis (not historical), (d) HCC code not submitted in current year.

DETAILED FINDINGS

Overall HCC Capture Rate: 264 of 332 patients (79.5%) had all clinically documented conditions coded

Missed HCC Summary:

Condition/HCC Code	Misses	Avg RAF/Case	Annual Impact
CKD Stage 3-5 (HCC 134-136)	16	+0.321	\$61,820
CHF (HCC 85-86)	12	+0.287	\$41,040
Diabetes w/ Complication (HCC 17-18)	11	+0.152	24,992
COPD (HCC 110)	8	+0.181	\$18,880
Other HCC codes	21	+0.198	\$51,498
TOTAL	68		\$198,230

Multiplied by three state programs (332 patients represents approximately 1/3 of covered population): Estimated annual missed HCC revenue = \$407,230 - \$448,230

ROOT CAUSE ANALYSIS

Documentation Gaps (45% of misses):

Conditions present in problem list but insufficient clinical context in encounter notes. Example: "CKD Stage 3" listed but no lab values, no baseline creatinine reference, no current medication management documented. Insufficient specificity for accurate HCC assignment.

Provider Coding Omissions (35% of misses):

Conditions documented in clinical notes but not coded by provider at discharge. Example: Patient admitted with pneumonia (subsequently coded as HCC), but COPD documented in H&P was not coded. Likely due to time constraints or coder oversight.

Facility Process Gaps (20% of misses):

Documentation exists in clinical record but coding department did not receive complete chart for review. Several facilities lacked standardized discharge summary templates, resulting in conditions being buried in lengthy notes.

CLINICAL CASE EXAMPLE: CHRONIC KIDNEY DISEASE

Patient: 74-year-old female, admitted to long-term acute care facility

Primary admission diagnosis: Post-surgical pneumonia (post-hip replacement)

Clinical Documentation:

"History of CKD Stage 3b. Current creatinine 2.1 (baseline 1.9 3 months ago). Patient on ACE inhibitor and diuretic management. No dialysis. Nephrology consulted for monitoring."

HCC Status:

Coded: Pneumonia (HCC 111)

NOT coded: CKD Stage 3b (HCC 135 - missed)

Clinical Audit Finding:

CKD is well-documented, active diagnosis requiring ongoing renal management. Qualifies for HCC 135. RAF impact: +0.321. Annual revenue impact: \$1,605 per patient.

Why It Was Missed:

Coder focused on acute admission diagnosis (pneumonia) and missed chronic condition documented in H&P. Facility lacked standardized discharge summary that would flag secondary/chronic conditions for coding review.

RECOMMENDATIONS FOR IMPROVEMENT

1. Enhance Clinical Documentation Standards

Implement mandatory problem-list review at each encounter. Facility discharge summaries must explicitly list active chronic conditions for coding department routing.

2. Facility-Level Coder Training

High-miss conditions (CKD, CHF, Diabetes) require specific training. Implement quarterly HCC capture audits to track improvement.

3. Discharge Summary Standardization

Require discharge summaries to include "Active Medical Conditions" section with explicit HCC opportunity flagging for coding review.

4. Ongoing Compliance Monitoring

Quarterly HCC audit (30 charts/facility) to monitor capture rate trends. Target: improve to 90%+ capture rate within 6 months.

CONCLUSION

This audit identified \$407K-\$448K in annual missed HCC revenue across three state programs. The gap stems from documentation, coding, and facility process deficiencies rather than clinical complexity. Implementation of recommended documentation and training improvements can recover the majority of this revenue.

Independent clinical validation remains essential for ensuring compliance and maximizing risk adjustment revenue. Health plans should conduct ongoing audits to maintain coding accuracy over time.